

CONTRACTOR ENGAGEMENT AGREEMENT

THIS AGREEMENT made this ____ day of _____ 2026

BETWEEN:

GOLDEN RATIO CLINICS PTY LTD

ACN 697 157 565

of Level 14, 60 Margaret Street, Sydney NSW 2000

(hereinafter called "the EMPLOYER")

AND:

DR DANIEL DEPP

(Qualifications: _____)

AHPRA Registration No., Medicare Provider No. and Prescriber No.: as recorded by the Contractor in the execution block at the end of this Agreement

(hereinafter called "the CONTRACTOR")

WHEREAS:

The Employer wishes to engage the Contractor as an independent medical practitioner on a part-time sessional basis for a trial period of three (3) months, subject to the terms and conditions herein.

NOW IT IS HEREBY AGREED AS FOLLOWS:

1. ENGAGEMENT AND COMMENCEMENT

1.1 The Employer engages the Contractor as a sessional medical practitioner on a part-time basis, commencing on _____ (the "Commencement Date"), on the terms and conditions set out herein.

1.2 The Contractor accepts this engagement and agrees to perform the duties outlined herein in accordance with all applicable laws, regulations, and professional standards.

1.3 This is a TRIAL PERIOD ENGAGEMENT of three (3) calendar months from the Commencement Date (the "Trial Period"), after which either party may terminate this agreement with one (1) week's written notice. Post-trial, standard notice periods of two (2) weeks shall apply.

1.4 Following the Trial Period, any continuation of engagement shall be subject to new contract discussions and mutual written agreement. There is no automatic conversion or renewal beyond the Trial Period.

2. ROLE AND RESPONSIBILITIES

2.1 The Contractor shall provide medical services as a qualified medical practitioner at the Employer's premises located at Level 14, 60 Margaret Street, Sydney NSW 2000, and such other locations as the Employer may reasonably designate, and by telehealth where clinically appropriate.

2.2 ROSTERED HOURS: The Contractor shall make themselves available for rostered clinical sessions totalling no more than ten (10) hours per week (the "Weekly Maximum"), as follows:

- (a) The days, start times and duration of each session shall be mutually agreed and confirmed in writing prior to the Commencement Date;
- (b) The Employer may adjust the roster within the Weekly Maximum by giving two (2) weeks' written notice;

- (c) The Contractor shall not cancel or reschedule a rostered session without prior written approval from the Employer;
- (d) The Weekly Maximum is a ceiling on the hours the Contractor is required to make available. It is not a commitment by the Employer to roster that number of hours in any week, and creates no entitlement to be rostered.

2.2A ADDITIONAL HOURS: The Employer may from time to time offer the Contractor hours in excess of the Weekly Maximum, and the Contractor may accept or decline any such offer.

- (a) Additional hours are worked only where the Employer requires them and the Contractor is willing and available to work them;
- (b) Each arrangement for additional hours shall be agreed in writing, which may be by email, on each occasion;
- (c) Neither party is obliged to offer, or to accept, additional hours, and a refusal by either party is not a breach of this Agreement and shall not of itself be grounds for termination;
- (d) Consultations conducted during additional hours are remunerated at the rate in Clause 3.1 without variation;
- (e) Where additional hours are worked regularly, the parties may agree in writing to vary the Weekly Maximum in accordance with Clause 17.1.

2.2B The Contractor is remunerated per Consultation in accordance with Clause 3 and not by reference to rostered hours. Rostered hours define the Contractor's availability for patient bookings; they do not create any entitlement to a minimum payment, a minimum number of Consultations, or payment for time during which no Consultation is conducted.

2.3 SCOPE OF CANNABIS PRESCRIBING: The Contractor, as a qualified General Practitioner Authorised Prescriber, is authorized to prescribe medicinal cannabis for all cannabis-related therapeutic indications, subject to:

- (a) Compliance with all TGA regulatory requirements and Royal Australian College of General Practitioners (RACGP) guidelines;
- (b) Adherence to the Employer's clinical protocols and patient eligibility criteria;
- (c) Maintenance of proper documentation and record-keeping;
- (d) Consultation with clinic management on complex or novel cases.

2.4 ZEDMED SYSTEM COMPLIANCE: The Contractor shall:

- (a) Utilize the ZedMed electronic patient management system for all clinical consultations, prescriptions, and patient records;
- (b) Use the system to the best of their professional ability and in accordance with Employer's protocols;
- (c) Maintain accurate, contemporaneous entries in patient records within ZedMed;
- (d) Comply with all ZedMed data security and access protocols;
- (e) Not circumvent or bypass ZedMed procedures;
- (f) Report any system issues or errors to clinic management immediately.

2.5 Professional Standards: The Contractor shall:

- (a) Provide professional medical services consistent with professional best practice, applicable legal requirements, and clinic operational standards;

- (b) Maintain all professional registrations, indemnity insurance, and qualifications in current standing;
- (c) Comply with clinic administrative policies and procedures necessary for clinic operations;
- (d) Keep accurate clinical records in accordance with applicable legal and professional requirements;
- (e) Maintain confidentiality and comply with all privacy legislation;
- (f) Deliver professional medical services with appropriate duty of care to all patients.

2.6 The Contractor:

- (a) Shall not represent themselves as an owner, partner, or shareholder of the Employer's business;
 - (b) May provide medical services to private patients outside clinic hours, provided this does not interfere with clinic commitments or breach confidentiality obligations.
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3. REMUNERATION

3.1 The Employer shall pay the Contractor for services rendered at the rate of AUD \$60.00 (plus applicable GST) per Consultation.

3.2 DEFINITION OF CONSULTATION: For the purposes of this Agreement, a "Consultation" means either an Attended Consultation or a Non-Attended Consultation, each as defined below, and both are payable at the rate in Clause 3.1.

- (a) ATTENDED CONSULTATION: a patient consultation that is:
 - (i) Booked through the Employer's booking system and attended by the patient, whether in person or by telehealth;
 - (ii) Conducted personally by the Contractor; and
 - (iii) Completed and documented contemporaneously by the Contractor in ZedMed, including any resulting prescription, in accordance with Clause 2.4.
- (b) NON-ATTENDED CONSULTATION: a booked appointment that remains on the Contractor's list at the commencement of the rostered session in which it falls, which the patient does not attend, and in respect of which the Contractor has completed and recorded the Contact Protocol in Clause 3.2A.
- (c) An appointment cancelled, rescheduled or released by the Employer before the rostered session commences is not a Consultation and no fee is payable for it.

3.2A CONTACT PROTOCOL: Where a patient does not attend a booked appointment, the Contractor shall, before that appointment may be treated as a Non-Attended Consultation:

- (a) Attempt to contact the patient at or about the scheduled appointment time, using the contact details and method recorded in ZedMed;
- (b) Make at least one further attempt to contact the patient during the same rostered session;
- (c) Return to the patient at the end of the consult list for that session and make a final attempt, so that not fewer than three (3) attempts are made in total, at separate times;
- (d) Where the patient responds and can be seen within the rostered session, conduct the Consultation, in which case it is an Attended Consultation and not a Non-Attended Consultation;
- (e) Record in ZedMed the time, method and outcome of each attempt.

A single unanswered attempt is not sufficient. Where the Contact Protocol is not completed and recorded, no fee is payable for that appointment.

3.3 The rate in Clause 3.1 is a flat rate per Consultation. It applies equally to every Consultation, whether an initial Consultation, a review Consultation or a re-script Consultation, and whether attended or non-attended, and does not vary with the duration or complexity of the Consultation.

3.4 NO PAYMENT WHERE REQUIREMENTS NOT MET: No fee is payable in respect of:

- (a) Any appointment the patient does not attend where the Contact Protocol in Clause 3.2A has not been completed and recorded;
- (b) Any Consultation not documented in ZedMed in accordance with Clause 2.4;
- (c) Time spent on administration, training, meetings, clinical governance activities under Clause 9, or handover under Clause 12, all of which form part of the engagement and are not separately remunerated;
- (d) Availability during a rostered session, or any part of one, in which no Consultation is conducted.

3.5 GST of 10% shall be added to all invoices submitted by the Contractor. The Contractor is responsible for remitting GST to the Australian Tax Office as required by law.

3.6 Payment shall be made monthly in arrears, on or before the 28th day of each calendar month, upon submission of a valid tax invoice detailing:

- Number of Consultations completed, identifying Attended and Non-Attended Consultations separately
- Dates of service
- ZedMed appointment or patient reference numbers (patient names shall not appear on invoices)
- Total amount payable (including GST)
- Contractor's ABN (if registered)

3.7 The Employer's ZedMed records are the record of Consultations completed for invoicing purposes. Any discrepancy between an invoice and those records shall be raised with clinic management within ten (10) business days of the invoice date and resolved by reference to the ZedMed record.

3.8 The Contractor shall be responsible for all superannuation contributions, tax withholding, and other statutory obligations as applicable under Australian taxation law. The Employer makes no superannuation contributions on behalf of the Contractor.

3.9 Payment shall be made by electronic funds transfer to the Contractor's nominated bank account. The Contractor shall provide valid banking details prior to the Commencement Date.

3.10 The Contractor shall submit invoices within ten (10) business days of the end of each calendar month.

3.11 No sick leave, annual leave, personal leave, or other paid leave entitlements apply to this sessional arrangement. Work missed due to illness or personal circumstances shall not be remunerated.

4. PATIENT OWNERSHIP AND RESTRICTION ON SOLICITATION

4.1 CLINIC PATIENTS: Patients who present to the Employer's clinic for services ("Clinic Patients") are registered with and billed through the Employer's clinic. The Contractor acknowledges:

- (a) Clinic Patients contact the clinic through the clinic's booking system and administrative processes;
- (b) The Contractor's role is to provide professional medical services to Clinic Patients within the clinic's framework;
- (c) The Contractor maintains professional clinical autonomy in delivering medical care consistent with applicable professional standards and guidelines;

(d) Patient records, contact information, and clinical data remain the property of the Employer's clinic.

4.2 PROFESSIONAL INDEPENDENCE: The Contractor:

- (a) Exercises independent professional judgment in clinical decision-making consistent with medical best practice and professional standards;
- (b) Is responsible for their own professional conduct and clinical outcomes;
- (c) Maintains full professional accountability as a registered medical practitioner with AHPRA;
- (d) Is not an agent or employee of the Employer but rather a medical professional providing services to the clinic's patients.

4.2A NO INFLUENCE ON CLINICAL DECISIONS: The parties acknowledge that the per-Consultation rate in Clause 3.1 is remuneration for professional services rendered and does not, and shall not be taken to, create any expectation, target, or incentive as to the number of Consultations conducted, the outcome of any Consultation, or whether any prescription is issued. The Contractor shall exercise independent clinical judgment in every Consultation, and shall conduct or decline to conduct, and prescribe or decline to prescribe, solely on clinical grounds.

4.3 RESTRICTION ON PATIENT POACHING - DURING ENGAGEMENT: During the term of this Agreement, the Contractor shall not, directly or indirectly:

- (a) Solicit or encourage any Clinic Patient to cease using the Employer's clinic for their medical services;
- (b) Use the clinic's patient contact information to contact patients for the purpose of directing them to alternative providers or the Contractor's own practice;
- (c) Represent to patients that they can or should see the Contractor outside the clinic's booking system;
- (d) Use access to clinic patient information obtained through this engagement to build a competing patient practice.

4.3A POST-ENGAGEMENT PATIENT CONTACT RESTRICTION: For a period of two (2) years following termination, the Contractor shall not:

- (a) Directly contact Clinic Patients encountered during this engagement for the purpose of soliciting them to transfer their care;
- (b) Provide cannabis prescribing services to Clinic Patients outside the Employer's clinic;
- (c) Otherwise use knowledge of Clinic Patients' identities to compete with the Employer's clinic.

4.4 PROTECTION OF PATIENT INFORMATION: All patient records, contact details, appointment schedules, treatment histories, and any identifying information relating to Clinic Patients are the exclusive property of the Employer. The Contractor shall not:

- (a) Copy, photograph, or remove any patient records or information;
- (b) Retain copies of patient data after termination;
- (c) Use patient information for any purpose other than providing services authorized by the Employer;
- (d) Disclose patient information to third parties except as required by law or Employer authorization.

4.5 NON-CIRCUMVENTION - MEDICINAL CANNABIS SERVICES: The Contractor acknowledges that the Employer's key service offering includes the provision of PGR-free medicinal cannabis prescriptions and related cannabis-based therapeutic services (hereinafter "Cannabis Services"). The Contractor shall not, directly or indirectly, during the term of this Agreement and for a period of three (3) years following termination:

- (a) REFERRAL RESTRICTION: Refer, direct, recommend, or facilitate any Clinic Patient to any other practitioner, clinic, or service provider for Cannabis Services or medicinal cannabis prescriptions of any kind;
- (b) ALTERNATIVE PATHWAY PROVISION: Provide, advise on, or recommend alternative pathways, providers, or mechanisms for Clinic Patients to obtain medicinal cannabis prescriptions or Cannabis Services outside the Employer's clinic;
- (c) CANNABIS PROTOCOL DISCLOSURE: Disclose, share, or communicate to any third party (including other practitioners, competitors, or patients) any information regarding:
 - (i) The Employer's medicinal cannabis prescribing protocols, treatment approaches, or methodologies;
 - (ii) The Employer's sourcing, supply, or access to PGR-free cannabis products;
 - (iii) The Employer's cannabis patient management systems or clinical pathways;
 - (iv) The Employer's cannabis-related business relationships or supplier arrangements;
- (d) COMPETITIVE CANNABIS PRACTICE: Establish, own, operate, or provide Cannabis Services through any other clinic, practice, or entity;
- (e) PATIENT FACILITATION: Facilitate, enable, or assist any Clinic Patient in circumventing the Employer's Cannabis Services through alternative practitioners or suppliers;
- (f) KNOWLEDGE EXPLOITATION: Use any knowledge, experience, contacts, or relationships gained through the Employer's cannabis practice to establish or develop competing cannabis prescribing services or to divert cannabis patients to other providers.

4.6 DURING-ENGAGEMENT RESTRICTION - CANNABIS CLINIC DUAL PRACTICE: During the term of this Agreement, the Contractor shall NOT work for, accept employment from, or provide services to any other clinic, practice, or entity that offers medicinal cannabis prescribing services or Cannabis Services. The Contractor may work for other general medical practices and non-cannabis clinics, but cannot hold concurrent engagement with another cannabis clinic.

4.7 ACKNOWLEDGMENT OF COMPETITIVE VALUE: The Contractor acknowledges that:

- (a) The Employer's PGR-free medicinal cannabis service is a core and distinctive business offering that provides competitive advantage;
- (b) Patient relationships developed through cannabis prescribing and treatment represent substantial business value to the Employer;
- (c) Breach of this clause would cause irreparable harm to the Employer's business, competitive position, and patient base;
- (d) Damages would be an inadequate remedy for such breach.

4.8 PATIENT POACHING - LIQUIDATED DAMAGES: The Contractor acknowledges that:

- (a) Each Clinic Patient represents substantial ongoing business value, estimated at AUD \$600 per month in prescription revenue, or AUD \$7,200 per patient per annum;
- (b) A Clinic Patient's average relationship duration is three (3) years, resulting in a per-patient lifetime value of approximately AUD \$21,600;
- (c) Loss of a Clinic Patient through Contractor solicitation or breach of non-solicitation provisions causes irreparable harm that is difficult to quantify;
- (d) As a genuine pre-estimate of loss, the Contractor shall be liable for liquidated damages of AUD \$20,000 per Clinic Patient stolen, diverted, or induced to transfer their care as a result of breach of Clauses 4.3,

4.3A, 4.4, 4.5, 4.6, or 4.7;

- (e) Liquidated damages are in addition to, and not in place of, the Employer's right to recover actual damages, consequential losses, lost profits, and the cost of patient re-acquisition;
- (f) The Employer may claim liquidated damages for each patient lost multiplied by the number of patients affected.

4.9 ENFORCEMENT: The Contractor acknowledges that breach of Clauses 4.3, 4.3A, 4.5, 4.6, or 4.7 would cause irreparable harm to the Employer's business and that the Employer shall be entitled to seek:

- (a) Immediate injunctive relief and restraining orders;
- (b) Liquidated damages as specified in Clause 4.8;
- (c) Actual damages and consequential losses;
- (d) All legal costs and enforcement expenses;
- (e) Damages without limitation.

The Contractor consents to the granting of injunctive relief to restrain breach of these provisions and waives any requirement for the Employer to post a bond or demonstrate irreparable harm beyond what is evident from the nature of the breach.

5. PROFESSIONAL INDEMNITY INSURANCE

5.1 The Contractor shall maintain professional indemnity insurance coverage throughout the term of this Agreement at a minimum level of AUD \$20,000,000 per claim, as is standard for medical practitioners in Australia.

5.2 The Contractor shall provide proof of current insurance prior to the Commencement Date and annually thereafter.

5.3 The Contractor shall not allow their insurance to lapse. Any gap in coverage shall constitute grounds for immediate suspension of clinical duties and may lead to termination.

5.4 The Contractor's insurance must cover services provided at the Employer's clinic and shall name the Employer as an interested party.

5.5 The Employer accepts no liability for medical negligence or professional conduct failures by the Contractor. The Contractor's insurance is the primary source of liability coverage.

6. PROFESSIONAL REGISTRATION AND QUALIFICATIONS

6.1 The Contractor warrants that they hold:

- (a) Current registration with AHPRA as a medical practitioner, under the AHPRA Registration No. recorded by the Contractor in the execution block at the end of this Agreement;
- (b) A current Medicare Provider Number and a current Prescriber Number with AUSPD, as recorded by the Contractor in the execution block at the end of this Agreement;
- (c) Current Authorised Prescriber status or equivalent authority to prescribe medicinal cannabis for the indications contemplated by Clause 2.3;
- (d) All other qualifications as stated in their application.

6.2 The Contractor shall notify the Employer immediately of any:

- (a) Investigations, complaints, or disciplinary actions by AHPRA;
- (b) Restrictions, suspension, or cancellation of registration;
- (c) Changes to professional qualifications or status;
- (d) Criminal proceedings or charges.

6.3 Failure to maintain required registrations shall automatically terminate this Agreement without notice.

6.4 The Employer reserves the right to verify all qualifications and registrations with relevant regulatory bodies at any time.

7. CONFIDENTIALITY AND PRIVACY

7.1 The Contractor shall maintain strict confidentiality regarding:

- (a) All patient information, medical records, and clinical data;
- (b) Clinic operations, procedures, financial information, and business practices;
- (c) Contractor information and clinic staff details;
- (d) Any proprietary information, systems, or processes used by the Employer.

7.2 Confidentiality obligations shall continue indefinitely, even after termination of this Agreement.

7.3 The Contractor shall comply with all privacy legislation including the Privacy Act 1988 (Cth) and any state-based health privacy laws.

7.4 Patient information shall only be accessed, used, and disclosed as necessary to provide authorized medical services or as required by law.

8. DATA PROTECTION AND CONFIDENTIALITY

8.1 PATIENT DATA CONFIDENTIALITY: The Contractor acknowledges that all patient data, medical records, and health information relating to Clinic Patients is strictly confidential and the exclusive property of the Employer. The Contractor shall maintain absolute confidentiality of all patient data, comply with the Privacy Act 1988 (Cth) and applicable state health privacy laws, and not disclose patient data to any third party except as authorized by the Employer or required by law.

8.2 PROHIBITION ON DATA SALE: The Contractor expressly acknowledges that patient data is NOT the Contractor's property and may NOT be sold, licensed, commercialized, or transferred to any third party for any purpose, including to data brokers, competitors, research organizations, or marketing companies. Any breach constitutes grounds for immediate termination and legal action.

8.3 RETURN AND DESTRUCTION OF DATA: Upon termination, the Contractor shall immediately return or destroy all patient records, data, and confidential information in all formats (digital, physical, or otherwise), and certify in writing compliance with this requirement. The Contractor shall not retain any copies of patient information.

9. CLINICAL GOVERNANCE AND QUALITY ASSURANCE

9.1 The Contractor shall:

- (a) Comply with all clinical governance standards, policies, and procedures established by the Employer;
- (b) Participate in quality assurance and clinical audit processes as required;

- (c) Submit to clinical peer review and performance evaluation;
 - (d) Maintain compliance with RACGP guidelines and TGA regulations for medicinal cannabis prescribing;
 - (e) Participate in continuing professional development relevant to cannabis therapeutics;
 - (f) Report to clinic management any clinical incidents, adverse events, or near-misses;
 - (g) Cooperate fully with any internal investigations or reviews of clinical practice;
 - (h) Maintain clinical records to standards required for audit and regulatory compliance.
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10. MANDATORY REPORTING OBLIGATIONS

10.1 The Contractor shall:

- (a) Comply with all mandatory reporting obligations under relevant legislation including the Health Practitioner Regulation National Law;
 - (b) Report immediately to clinic management any patient safety concerns, adverse events, or clinical incidents;
 - (c) Cooperate fully with AHPRA investigations or notifications;
 - (d) Report any complaints made against the Contractor to clinic management within 24 hours;
 - (e) Notify the Employer immediately of any investigation, suspension, cancellation, or restriction of registration;
 - (f) Maintain records of all mandatory reports and notifications.
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11. BREACH NOTIFICATION

11.1 The Contractor shall notify the Employer in writing within 24 hours of:

- (a) Any AHPRA inquiry, investigation, or complaint;
- (b) Any police investigation or criminal charges;
- (c) Any professional conduct allegation;
- (d) Any civil litigation or claim;
- (e) Any lapse or restriction of professional indemnity insurance;
- (f) Any loss or suspension of prescriber numbers or registration;
- (g) Any change in professional registration status;
- (h) Any event that could reasonably affect the Contractor's fitness to practice.

11.2 Failure to notify shall constitute a material breach and grounds for immediate termination.

12. PATIENT HANDOVER AND CONTINUITY OF CARE

12.1 Upon termination of this Agreement or at the Employer's direction, the Contractor shall:

- (a) Immediately cease all clinical work and patient consultations;
- (b) Participate in a handover process to ensure continuity of patient care;
- (c) Provide comprehensive handover notes for all active patients under the Contractor's care;
- (d) Ensure all patient records are complete, accurate, and up-to-date in ZedMed;

- (e) Transfer all patient information and clinical documentation to the Employer or designated successor practitioner.

12.2 HANDOVER PROCEDURES:

- (a) The Contractor shall complete a written patient summary for each active patient, including:
 - (i) Current diagnosis and treatment plan;
 - (ii) Medicinal cannabis prescription details (product, dosage, frequency, review dates);
 - (iii) Recent clinical notes and assessment outcomes;
 - (iv) Any outstanding investigations or referrals;
 - (v) Patient contact details and preferred communication methods;
 - (vi) Any special clinical considerations or patient preferences.
- (b) The Contractor shall participate in patient transition sessions where clinically appropriate, with the Employer's designated successor practitioner present;
- (c) The Contractor shall remain available for a reasonable handover period (minimum 5 working days) to answer questions about patient cases and clinical decisions;
- (d) All handover documentation shall be completed within 10 working days of termination date.

12.3 PATIENT COMMUNICATION: The Employer shall notify patients of the change in practitioner. The Contractor shall not:

- (a) Contact patients directly regarding the handover except as authorized by the Employer;
 - (b) Provide contact details or information to facilitate patients following the Contractor to another practice;
 - (c) Discuss the termination or reasons for departure with patients;
 - (d) Offer personal contact details or alternative care arrangements to patients.
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13. CONDUCT AND COMPLIANCE

13.1 The Contractor shall:

- (a) Conduct themselves professionally at all times;
- (b) Comply with clinic policies and procedures;
- (c) Cooperate with clinic management and administrative staff;
- (d) Not engage in conduct that brings the clinic into disrepute;
- (e) Attend on time and fulfil rostered sessions;
- (f) Maintain professional relationships with colleagues and staff.

13.2 The Contractor shall not:

- (a) Arrive for work under the influence of alcohol or drugs;
- (b) Engage in harassment, discrimination, or bullying;
- (c) Disclose confidential information;
- (d) Compete with the Employer's business;
- (e) Make public statements on behalf of the clinic without authorization.

13.3 Breach of this clause may result in immediate termination without notice.

14. TERMINATION

14.1 TRIAL PERIOD TERMINATION: During the Trial Period (first three months), either party may terminate this Agreement by providing one (1) week's written notice, without cause or penalty.

14.2 POST-TRIAL TERMINATION: After the Trial Period, either party may terminate by providing two (2) weeks' written notice in writing.

14.3 IMMEDIATE TERMINATION: The Employer may terminate this Agreement immediately and without notice if:

- (a) The Contractor fails to maintain professional registration or indemnity insurance;
- (b) The Contractor breaches any provision of Clauses 4 (Patient Ownership), 5 (Indemnity Insurance), 7 (Confidentiality), 10 (Mandatory Reporting), 11 (Breach Notification), or 13 (Conduct);
- (c) The Contractor is charged with or convicted of a criminal offence;
- (d) The Contractor's AHPRA registration is suspended or cancelled;
- (e) The Contractor engages in gross misconduct or professional incompetence;
- (f) The Contractor solicits or accepts Clinic Patients outside the clinic;
- (g) The Contractor works for or accepts engagement at another cannabis clinic while engaged by the Employer.

14.4 Upon termination, the Contractor shall:

- (a) Immediately cease all clinical work;
- (b) Return all clinic property, keys, access cards, and materials;
- (c) Return or destroy all patient records and confidential information;
- (d) Provide written confirmation of compliance with Clause 4 (Patient Restriction);
- (e) Not represent themselves as affiliated with the clinic;
- (f) Complete patient handover as detailed in Clause 12.

14.5 Fees for Consultations completed up to and including the termination date remain payable in accordance with Clause 3, subject to the Contractor submitting a valid tax invoice and completing the record-keeping required by Clauses 2.4 and 3.2.

14.6 Termination shall not relieve the Contractor of obligations under Clauses 4 (Patient Ownership), 7 (Confidentiality), and 8 (Data Protection), which shall survive indefinitely.

15. INDEPENDENT CONTRACTOR STATUS

15.1 The Contractor is an independent contractor and not an employee of the Employer for the purposes of the Fair Work Act 2009 (Cth), the Income Tax Assessment Act 1997 (Cth), or any other legislation.

15.2 The Contractor is responsible for:

- (a) All taxation obligations and PAYG withholding;
- (b) Superannuation contributions;
- (c) Workers' compensation insurance (if applicable);
- (d) Professional indemnity insurance;
- (e) Income protection and other personal insurances.

15.3 The Contractor is not entitled to leave, penalty rates, shift allowances, or other employee benefits.

15.4 The Employer does not provide workers' compensation coverage. The Contractor must arrange their own coverage if required by law.

16. LIABILITY AND INDEMNITY

16.1 The Contractor indemnifies and holds harmless the Employer from and against all claims, damages, losses, costs, and expenses arising from:

- (a) The Contractor's provision of medical services;
- (b) The Contractor's breach of this Agreement;
- (c) The Contractor's professional negligence or misconduct;
- (d) Any claims by Clinic Patients against the Contractor;
- (e) The Contractor's violation of any law or regulation.

16.2 The Contractor's professional indemnity insurance shall be the primary source of coverage for professional liability claims.

16.3 The Employer accepts no liability for the Contractor's actions, omissions, or professional conduct.

17. VARIATION AND DISPUTE RESOLUTION

17.1 This Agreement may only be varied by written agreement signed by both parties.

17.2 If a dispute arises, the parties shall attempt to resolve it through good faith negotiation.

17.3 If negotiation fails, the matter shall be referred to mediation under the Mediation Scheme of the Australian Medical Association or equivalent dispute resolution process.

17.4 If mediation does not resolve the dispute, either party may refer the matter to legal proceedings.

18. GOVERNING LAW

18.1 This Agreement shall be governed by and construed in accordance with the laws of New South Wales, Australia.

18.2 The parties submit to the exclusive jurisdiction of the courts of New South Wales.

19. ENTIRE AGREEMENT

19.1 This Agreement constitutes the entire agreement between the parties and supersedes all prior negotiations, representations, and agreements.

19.2 If any clause is found to be invalid or unenforceable, the remaining clauses shall continue in full force and effect.

19.3 The Contractor confirms they have read this Agreement, understand its terms, and have had the opportunity to seek legal advice before signing.

SIGNED as an AGREEMENT:

EXECUTED by GOLDEN RATIO CLINICS PTY LTD (ACN 697 157 565) in accordance with section 127(1) of the Corporations Act 2001 (Cth):

By: Quentin Leon Tetz

Name: Quentin Leon Tetz

Title: Director

Date: 16 September 2026

By: Jullian James Westlake

Name: Jullian James Westlake

Title: Director

Date: 16 September 2026

COUNTERSIGNED for the Employer:

By: Maximillian Matthews

Name: Maximillian Matthews

Title: Operating Partner

Date: 16 September 2026

DR DANIEL DEPP

Signed: _____

Name: _____

Date: _____

AHPRA Registration No.: _____

Medicare Provider No.: _____

Prescriber No.: _____

END OF AGREEMENT